CHAPTER 17



Community Psychology: Evolution and Orientation

The Third Mental Health Revolution

In the minds of many clinicians, we are entering a new era in the understanding and treatment of mental disorders. Heralded as a "third mental health revolution" (Hobbs, 1964) or a "third psychiatric revolution" (Bellak, 1964), the emergent field of community mental health (community psychology, community psychiatry) is proclaimed as being as radical a change in perspective on human malfunctioning and its alleviation as was wrought in earlier times by Pinel, when he struck the chains from the insane, and by Freud, when he showed that neuroses are psychologically determined and curable through therapeutic conversations. As consequence of the first revolution, the mentally disturbed emerged as sick people worthy of humane concern; from the second, their conditions were conceived as psychologically determined and psychologically treatable. The thrust of the third mental health revolution lies in the quest for the prevention of emotional disorders through social and community interventions aimed at their social determinants. Even when viewed more modestly, the hope of the community movement is that necessary services can be made more effective and available for the total population, including the poor and the alienated who have benefited least from prevailing practices.

Policy makers join with many mental health professionals in the conviction

that new models of mental health must evolve if the needs of all of the people are to be served. At the same time, thoughtful critics have cautioned against the dangers of overenthusiastic acceptance of the "newest therapeutic bandwagon" (Dunham, 1965; Kubie, 1968). Work in the field has been motivated largely by pragmatic and idealistic concerns, with the development of the necessary base of theory, systematic knowledge, and program-relevant research thus far lagging behind.

In this and later chapters, we will consider the origins and background of community psychology and the community mental health movement, the emerging conceptual framework and some of the newer developments in mental health practices and techniques conceived within a community framework. Any account at this point is necessarily incomplete as the field is in a state of vigorous growth. Increasing numbers of psychiatrists and psychologists are characterizing themselves with the qualifying adjective "community." A reference guide to papers and books in the field published in 1969 listed 1,500 references which appeared in the 1960s (Golann, 1969); by now, the number has surely more than doubled. In 1965, the *Community Mental Health Journal*, exclusively devoted to work in the field, was started; in 1973, two new journals of community psychology were launched. Also in 1965, a conference was held near Boston to consider issues in the training of psychologists for work in the community mental health field. At that conference, the term "community psychology" emerged to describe the field which included, but also extended beyond, the community mental health roles of clinical psychologists. A Division of Community Psychology was added to the American Psychological Association, which now has over a thousand members. Though containing diverse orientations and activities, a number of broad orienting principles characterize community psychology.

Principles of Community Psychology

Here are some of the major themes which characterize current thinking in community psychology (see M. Levine, 1969; Roen, 1971; Cowen, 1973; Bloom, 1973; Zax and Specter, 1974).

1. Social-environmental factors are critically important in determining and changing behavior.
2. Social and community interventions (*system-oriented* interventions as against *person-oriented* interventions) can be effective for making social institutions (e.g., family, school) more health-enhancing as well as for reducing individual suffering.
3. Such interventions should be aimed at prevention rather than treatment or rehabilitation of emotional disorders. Not only the individual-in-need but the population-at-risk is the proper concern of community psychology.
4. Intervention should have as its goal the enhancement of social competence rather than simply the reduction of psychological distress. Community-oriented programs should stress the adaptive rather than the pathological in social life.
5. Help is most effective when available close to the settings in which problems arise. Therefore, community clinicians should work in familiar settings close to the person in need rather than in socially and geographically alien settings.

6. The community clinician should reach out to clients rather than waiting passively for them to seek his services. Such services should be flexible, readily available at the place and time of need, and offered in an atmosphere which reduces rather than accentuates the social distance between helper and helped. Help should be available to those who need it most, not only to those who seek it.
7. To use available resources and to extend his potential impact, the professional should collaborate with community resource people (caretakers) and use nonprofessional co-workers. His work may involve consultation more than direct services.
8. Traditional role requirements and professional customs have to be relaxed. Community services require imaginative programming and new conceptual models; innovation is to be encouraged.
9. The community should participate in, if not control, the development and operation of programs which are to serve its needs. Program priorities should reflect the needs and concerns of community members.
10. Mental health problems should be broadly rather than narrowly viewed, for they interlock with many other facets of social well-being, such as jobs, housing, and education. To be maximally effective, community mental health programs should deal with as wide a range of social problems as possible.
11. Educating the public to understand the nature and causes of psychosocial problems and the resources available for dealing with them is a valued task.
12. Since many mental health problems relate to broad-scaled social stresses such as poverty, racism, urban density, and alienation, which are beyond the reach of professional interventions, the community psychologist should be oriented toward, and as possible facilitate, social reform.
13. To develop the knowledge necessary for informed intervention, community psychology requires naturalistic and ecological research approaches.

The Field of Community Psychology

Community psychology is oriented toward averting human problems rather than simply repairing those which already exist. The ultimate goal of prevention, community psychologists believe, can best be attained through social and community interventions which alter the social institutions which vitally affect the patient's well-being. Toward this end, emphasis is shifted from intra-individual factors, whether psychodynamic or biological, toward greater attention to the social and institutional determinants of human functioning and disorder. Community psychology represents a "public health" rather than a "treatment" approach to mental health problems; consequently, there is greater concern with populations and prevention than with individuals and cure.

In actuality, however, community psychology covers a broad range of activities. It is more a way of thinking than an explicit set of principles and practices. As we noted in our earlier discussion of alternate models of mental health intervention (Chapter 5), the community approach spans a spectrum from interventions close to the "therapeutic" mode to those of "social action." In surveying a number of community-oriented programs, Schon (1968) notes four groups: (1) those which extend existing practice, aiming at populations for whom treatment has been relatively inaccessible; (2) those which focus on secondary prevention, to identify mentally ill individuals before their illness has reached crisis propor-

tions; (3) those which aim to influence the mental health climate of a community through consultation to key institutions such as courts or schools; and (4) those in which the unit of treatment itself shifts from individuals or families to the community itself, and from the distress of individuals to the sickness of communities. Thus, in practice one community psychologist may work in direct, therapylike services to people presently in distress (as in crisis intervention, suicide prevention, etc.) while another takes as his target the social setting itself, in the interest of people living under its influence. An example of the latter might be an effort to alter the social structure of a school, and hence the behavior of teachers, as to "create a setting" which is more likely to enhance the emotional and intellectual growth of children in it (Sarason, 1972).

The term "community psychology" itself was born in the Boston conference of 1965 (Bennett, 1965; Bennett et al., 1966). The participants were psychologists in community mental health programs, but they saw a new field emerging which extended beyond concern with the mentally ill and therefore required a new name. In their view, community psychology should work toward fostering the adaptive functioning of large groups of people, only a limited number of whom could be considered actual or even potential psychiatric patients. Community psychologists would serve as ". . . change agents, social systems analysts, consultants in community affairs, and students generally of the whole man in relation to all his environments" (Bennett, 1965, p. 833). Some held that community psychologists should go beyond consultation to a social activist role; they should seek to control power as well as influencing those who now wield it. All agreed, however, that the community psychologist was to be a "participant conceptualizer" who could use his scientific skills to generate relevant knowledge and conceive programs within a model of action research. Though most were trained clinicians, community psychology was viewed as a parallel but separate field from clinical psychology, addressing distinct problems, involving different concepts and techniques and requiring separate training. Since the Boston conference, the term "community/clinical psychology" has emerged to characterize the border area, in which clinicians use socially oriented concepts and methods. This is the major area of our concern in this volume, but for simplicity I will use the term community psychology without qualification. In the present context, therefore, community psychology can be viewed as an orientation within the broadly defined field of clinical psychology, though in other respects it is a field in its own right.

Community Psychology, Community Mental Health, and Related Concepts

The term "community psychology" overlaps with the older concepts of community psychiatry, preventive psychiatry, community mental health and social psychiatry, all of which emphasize the importance of the social environment in determining and changing human behavior. Each of these terms has been defined in more conservative or more radical ways, as either being chiefly concerned with the development of new methods for the care of the mentally ill or being oriented

to broad-scaled social change of importance to the normal as well as deviant members of society. Some have distinguished a practice field (community psychiatry) from a field of research and theory (social psychiatry) (e.g., Zusman, 1970). The term "social psychiatry," it is interesting to note, dates at least from 1917 when Southard first coined it in analogy to the then new field of social psychology (N. W. Bell and Spiegel, 1966). Just as "social psychology" comes from the conjunction of social and psychological concepts, Southard believed that "social psychiatry" should join social and psychiatric concerns. Terminological complexity is added by the fact that sociologists have had a long-standing interest in the study of the action of social factors on emotional disease. Thus Dunham (1947), reviewing this field for the American Sociological Society, described an already well-developed area of theory and research. "A field of 'social psychiatry' has emerged," he notes, "from the attempt to study certain problems considered as psychiatric from the point of view and with the techniques of the sociologist." However, he is troubled by the "semantic difficulty" in the term since other social and behavioral scientists are much involved in the same area. This fact, he notes further, "questions the proposition that a field of social psychiatry should be regarded as exclusively the product of the sociologist" (Dunham, 1947, p. 183). Indeed, the term "social psychiatry" is widely used by psychiatrists, by some to characterize a focus of practice as well as research (M. Jones, 1968). Thus to avert confusion, Weinberg (1967) urges other sociologists to use Arnold Rose's term "psychiatric sociology" to identify their unique concerns. The same general area has been called the "social psychology of mental health" by psychologists such as Wechsler, Solomon, and Kramer (1970). Clearly, there are no distinct or consistent meanings implied in the various combinations of "social," "community," and "sociological" with "psychiatry," "psychology," and "mental health." In addition, the terms favored often reflect the disciplinary allegiance of the user.*

In this regard, the term "community mental health," which has gained wide usage in recent years, may be the most neutral and inclusive. It can properly include the work of all of the "helping professions," such as clinical psychologists, psychiatrists, psychiatric social workers, group and youth workers, school and counseling psychologists, welfare workers, and criminologists and probation officers. The same term can encompass theory and research as well as practice techniques and programs. Although "community" does not imply as broad a focus as "social," in the present usage the two terms have become essentially synonymous. Community mental health can be narrowly conceived to mean primarily the application of socially oriented treatment techniques to mental patients but it can as much be broadly conceived to include prevention and efforts to foster psychosocial competence and positive mental health. Thus, the staff of the Langley Porter Neuropsychiatric Institute of the University of California in San Francisco defined community mental health as follows:

> The broad, multidisciplined field concerned with the wide variety of forces and structures in a community which affect the emotional stability (positive growth, development, and functioning) of a significant group of its members. It is contrasted with the traditional *clinical* approach which focuses on the particular individual in emotional distress. The attention is directed at social institutions including those concerned with welfare, health, legislation, minority groups, employment, education, church, and their interactions which can in their functioning either enhance or hinder the emo-

* The student interested in the variety of meanings in these terms should read S. E. Goldston (1965), Weinberg (1967), Sabshin (1969), Srole (1968), and Zax and Specter (1974).

> tional growth of a large segment of the population [quoted in S. E. Goldston, 1965, pp. 197–198].

In this statement, "community mental health" is essentially synonymous with "community psychology," as defined earlier in this chapter. For many, however, community mental health has come to mean the provision of psychiatric services in community mental health centers. To preserve the sense of the broader field which concerns us here, the term "community psychology" will be used, although as noted the distinctions between these terms are difficult to maintain.

Some Factors Underlying the Emergence of Community Psychology

Before going on to consider community psychological concepts and interventions in further detail, we should look more closely at the ideological, professional, and social climate which spawned the community orientation and gave it its particular philosophical flavor. Among the more salient issues are the discouragement with existing mental health concepts and functions, the changing concepts of mental illness and health, the manpower shortage, the deterioration of the state hospitals and the quest for alternatives, and the growing interest, in society at large, in problems of poverty and race in a general atmosphere of increasing social concern. Each of these matters will be considered in turn.

DISCOURAGEMENT WITH EXISTING MENTAL HEALTH CONCEPTS, ACTIVITIES, AND ROLES

Community psychology emerged, in fair measure, out of what Hersch (1968, 1969) has termed a "discontent explosion" among clinicians. Established beliefs about the nature of mental disorders and their treatment have been called into question; clinicians have found traditional roles frustrating; they question whether they can contribute significantly to the vast unmet needs of large segments of the population.

Disenchantment with Psychotherapy. Growing criticism from many quarters has shaken the faith of many clinicians in both the effectiveness and efficiency of traditional psychotherapies. Both humanistic and behavioral psychologists, from opposing vantage points, have questioned fundamental premises, as we saw in Chapter 14. More devastating, however, was the claim that psychotherapy had yet to demonstrate its effectiveness even as against no treatment whatever. This argument, launched by Eysenck in 1952, led to considerable controversy over the interpretation of the limited amount of research data available; in time, it inspired more sophisticated research studies. Although the presently available evidence shows psychotherapy to be considerably more effective than Eysenck claimed, as we concluded in Chapter 16, it is still true that it is less than the panacea some thought. However, the same body of research has brought out some genuine issues as to the universal applicability and value of psychotherapy. The ideal patient, it has been noted repeatedly, is someone already on the way to psychological

health, with a fair degree of psychosocial competence, intelligence, personality integration, and insight. The more severely disturbed, the mentally retarded, the unmotivated and many of those facing the stresses of poverty or race are less well served.

Even if there were no question of the effectiveness of psychotherapy, it is clearly not an efficient procedure. It is costly in time, effort, and money, and hence most available to the most motivated and affluent. During a professional lifetime, a clinician practicing intensive psychotherapy can see only a very limited number of people. The consciences of many concerned psychoanalysts have long been troubled by this fact, however much they may help the few patients they treat. Particularly in a time of heightened social need and limited professional manpower, the need for increasing the efficiency of treatment becomes more acute. Almost seventy years ago, Freud predicted at the Fifth International Congress of Psychoanalysis in 1918 that the time had arrived for "the conscience of the community" to awaken to the fact that "the poor man has just as much right to help for his mind as he now has to the surgeon's means of saving life. The task will arise," Freud continued, "for us to adapt our technique to the new conditions" (quoted by Galdston, 1971).

Despite many statements of concern and efforts at innovation, the problem of adapting psychotherapy or finding substitutes for it remains with us. Thus, Eisenberg (1962a) speaking out vigorously for a need to shift from a therapeutic to a preventive orientation notes: "The limitations of present therapeutic methods doom us to training caretakers at a rate that ever lags behind the growing legions of the ill, unless we strike out successfully in new directions in the search for cause and treatment. . . . Society can ill afford today's precious overspecialization in which trainees may learn one method even superbly well but remain abysmally unaware of the problems besetting the bulk of the mentally ill" (p. 825).

Changing Concepts of Mental Health and Illness. More broadly, there has been widespread dissatisfaction with traditional "medical" concepts, particularly as reflected in the "custodial" but also in the "therapeutic" approaches to mental problems (see Chapters 4 and 5). From various perspectives, it has been argued that conceiving emotional disorders in analogy to physical diseases limits our understanding of human problems and our effectiveness in alleviating them. Of particular concern is the implication, in the medical model, that mental diseases reside in the individual and hence that intervention must involve treatment of the sick person to the end of removing or altering the pathological processes within him. In opposition to such a view, many have argued the need for a "social," "preventive," "growth-and-development" or in the general term being used here "community psychology" point of view which sees human problems as residing in the interplay between the distressed person and social forces.

At the extreme are those who hold that "mental illness is a myth" (Szasz, 1961) and that mental illness can be conceived essentially as a social role which is adopted by the individual responding to the attributions of relevant others in his social environment (Scheff, 1966, 1974; Sarbin, 1969). This point of view has been described as labeling theory, since the emphasis is shifted from disease-producing factors within the person to the social definitions of his society. A person is "psychologically sick," this view holds, to the extent that his deviant acts are so labeled by relevant and powerful agents of society. In its most simple and direct form, the labeling theory oversimplifies greatly for it negates much of what

is known about psychological as well as biological factors in emotional disorders. But it does call attention sharply to those factors in the social environment that, if not causative, are critically important in maintaining and exacerbating mental disorders. As Sanford (1965) and others have pointed out, social forces do not operate in a vacuum; rather they act on an existing personality with its distinctive structures and functions.

Dissatisfaction with Existing Professional Roles. Mental health professionals of all kinds, like their critics, have become concerned with their limited ability to effect important changes in the level of psychological distress in the larger community. Working in clinics, private offices, and hospitals, waiting rather passively for troubled people to present themselves for help, many have come to realize the need for a greater area of influence and for earlier, quicker, and more effective services. Clinical psychologists face the additional problem of feeling unfulfilled in medical settings where primary decisions are made by physicians. The medical environment, many feel, provides too little opportunity to exert their independence and competence; too often, they see themselves as second-class citizens in a mental health enterprise dominated by medical men. The quest for autonomy has led many psychologists into the private practice of psychotherapy. Many others, however, see the new roles emerging in community psychology as providing a better route toward professional autonomy and allowing greater freedom to experiment with uniquely psychological interventions.

THE MANPOWER SHORTAGE

If there is one incontrovertible fact in the field of mental health today, it is that there is a serious and growing gap between manpower resources and the public's needs, at least when the major mental health professions are considered in their present roles and functions. The blatant manpower shortage has been a major impetus to reconceptualize professional roles, to suggest alternate ways of deploying professional time, and to seek new workers in subprofessional roles.

Albee (1959), in one of the major studies of the Joint Commission on Mental Illness and Health, surveyed manpower trends in the major mental health professions. His findings led the author and the Joint Commission to conclude "with frank pessimism, that sufficient professional personnel to eliminate the glaring deficiencies in our care of mental patients will ever become available if present population trend continues without a commensurate increase in the recruitment and training of mental health manpower." And they are not sanguine about such increases taking place. It is pointed out that in medicine itself, the parent field of psychiatry, there is a similar deficit between the number of physicians needed and those being trained. Despite the rapid growth of psychology during the late 1940s and 1950s, particularly in the clinical area, the number of available psychologists also lags behind. The fields of social welfare, psychiatric nursing, and occupational therapy show similar shortages. Private practice was absorbing increasingly large numbers of clinicians, and courts, schools, prisons, and other institutions were making increasing use of them. Career opportunities in college teaching and research centers have also attracted larger numbers of psychologists away from clinical service roles. Thus, relatively few clinicians were available for work in public psychiatric hospitals, an issue which particularly concerned the Joint Commission. At that time, three-quarters of the budgeted

positions for psychologists and physicians in state and county hospitals could not be filled. Reviewing the manpower picture some years later left Albee (1967) equally pessimistic about future prospects.

The emerging community mental health centers of the 1960s have had difficulties in finding sufficient numbers of adequately trained personnel. As the hospitals they sought to complement, they have had to compete with a number of attractive alternatives for the already short number of trained professionals. In one projection, the recommended staffing for the new community mental health centers was one psychiatrist, one clinical psychologist, and one psychiatric social worker for every 50,000 people. With our present national population, this would mean that an additional 14,000–15,000 professionals would have to be trained in order to meet current needs.

To make matters worse, manpower authorities call attention to the fact that demand tends to increase with increasing supply (Arnhoff, 1968). It has been shown, for example, that countries differ enormously in the hospital resources available for mental patients, and in every case the beds are fully used. Thus, India had only 0.1 bed per 100,000 population (as of 1959) while, at the other extreme, Ireland and New Zealand had over 450 beds per 100,000 people (Lin, 1968). This hardly means that the Irish and New Zealanders have 4,500 times more psychotics than the Indians. Rather, it reveals the greater economic resources and social concerns of these countries which provides needed beds which are then used. Similarly, the number of psychiatrists, Lin also notes, differs among countries over a range of 250 to 1, between the best endowed and least well endowed nations.

With increased affluence, reduced public resistance, and greater readiness to seek help for less crippling psychological problems, demand for psychotherapy and other mental health services will predictably continue to rise. Moreover, new programs of federal and private health insurance, and contributions by companies and labor unions, will help reduce the financial burden of such care. It is fully predictable that a greater proportion of the population, in the future than in the past, will seek psychological help. Even if the number of professionals could be radically increased, the demand could rise to meet the supply of available time.

In view of present needs, population growth, the disproportionate growth of demand for psychological services, and the limited capacity of training institutions, it is easy to see why the manpower problem has spurred the search for new sources of manpower, more effective utilization of available professional time, and new conceptual models for working with human problems.

POVERTY AND MENTAL HEALTH

When one looks at the problems of the poor and minorities the need for new services comes especially into sharp focus. The life conditions of the poor are pathogenic (Sanua, 1961; Hollingshead and Redlich, 1958; Riessman, Cohen, and Pearl, 1964; Fried, 1964b; Srole et al., 1962; among others). Granted, we still do not know enough about the intervening links between economic deprivation, its sociological expressions, personality development, and subsequent pathology, the overall picture of greater incidence and prevalence of both gross pathology and lessened psychological competence in the culture of poverty is all too well established.

Not only is there greater need but there are also great inequities in the

deliveries of services to the poor. Hollingshead and Redlich (1958) showed in their classic study that the poor tended to end up in custodial hospitals while the well-to-do were treated in private psychotherapy. Summing up, M. B. Smith and Hobbs (1966, p. 14) noted that "the more advanced mental health services have tended to be a middle-class luxury; chronic mental hospital custody, a lower-class horror. The relationship between the mental health helper and helped has been governed by an affinity of the clean for the clean, the educated for the educated, the affluent for the affluent." This is not to suggest that mental health workers have maliciously decided to ignore the needs of masses of the population; indeed, most in their personal attitudes would sincerely wish to see the state of the poor improved. But the conditions of clinical work, the settings in which it takes place, the nature of the contract between helper and helped all lead to the likelihood that disproportionately more service will be given to the middle-class and disproportionately less to the poor. The psychological-mindedness, introspectiveness, striving for personal growth, and motivation for therapy, important to psychotherapy, are part of the middle-class values shared by the therapist and his middle-class patient. Although, as we saw in Chapter 16, evidence does not support the simple assumption that psychotherapy is ineffective with poorer clients, it remains true that it is more difficult. When dealing with the poor, therapists and patients often speak different languages, sometimes literally as well as figuratively, and work toward different ends.

On his part, the lower-class person may distrust the mental health effort. Its institutions, hospitals, and clinics, are part of the white, affluent establishment, as are most of its practitioners. As Reiff, Riessman, and others have pointed out, the poor person often sees his problems differently than does the middle-class client. Psychological distress, he feels, is less due to his personal inadequacies than to the harsh conditions imposed on him. His problems do not reside within, but arise from without. He is oriented to action, expects to see things done, and has little patience for talk. The middle-class patient, like his therapist, sees himself handicapped by neurotic symptoms which cause pain and limit effectiveness; self-understanding and increased personal freedom are the proper goals of therapy. By contrast, the lower-class person suffers more from helplessness in the face of debilitating social stresses and his lack of competence to deal with them. Reiff (1966) aptly notes that, as they see themselves, disturbed middle-class patients are *victims of their own selves* while lower-class patients see themselves as *victims of circumstances*. For the former, *self-actualization* may be a proper goal; for the latter, *self-determination* is more realistic and meaningful (Reiff, 1966a). Intervention, to be maximally useful to the poor client, must be designed to facilitate new social competencies as well as release existing ones from neurotic inhibitions.

Not only in the field of mental health, but in health services generally, black Americans and other minorities have great unmet needs, the more so when they are viewed in the light of the health advances in the majority population. Thus, while rates of infant and maternal mortality have fallen in the United States over the past fifty years in all segments of the population, the discrepancy between blacks and whites increases. Before 1920, blacks had 79 percent more infants stillborn than whites; by 1963, according to a National Health Survey, it was 320 percent higher (English, 1967). Correspondingly, the postnatal mortality rate among black infants was 80 percent higher than for Caucasians in 1915–1919; by

1963, it was 180 percent higher. Medical services are usually less adequate and they tend to demean their poor clients. From his experiences in the public clinics and hospitals of New York City, Alonzo Yerby once expressed the hope that America could so organize its health system that all people would have "equal access to health services as good as we can make them, and that the poor will no longer be forced to barter their dignity for their health" (quoted by J. T. English, 1967). Community mental health services are intended as a step toward this end.